ISSUED, VALID FROM 30 SEPTEMBER 2026

Wegovy (semaglutide) Tablets for Weight Management

Patient Group Direction, version 013, issued 30 September 2026. Oral semaglutide 1.5mg, 4mg, 9mg and 25mg. Adults 18 to 85.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Initial assessment

Before commencing treatment the patient must be assessed by a registered healthcare professional. The initial assessment should be face to face and should include, but need not be limited to:

- Causes of weight gain. Refer to the GP if a prescribed medicine is causing weight gain.
- The patient's lifestyle, diet and level of exercise.
- Whether the patient has tried to lose weight before, and what has and has not worked.
- Other factors that may be contributing, including mental health, environmental and psychological factors. Consider signposting to another health professional.
- Other disease states that may predispose to weight gain. Consider referring to the GP.
- The patient's expectations of weight loss, and whether they are realistic.
- **Height, weight and BMI. RECORD THE WEIGHT AS THE BASELINE. Set a realistic target weight and agree review intervals.**

GET REAL HEALTH

Wegovy (semaglutide) tablets

Summary of NICE guidance and the SmPC for Wegovy tablets (oral semaglutide) for weight management in adults
Wegovy 25 mg tablets, Summary of Product Characteristics, Novo Nordisk Limited, last updated on emc 16 June 2026 (medicines.org.uk product 102347), sections 1 to 4.9 and 5.1 read; the same SmPC covers the 1.5 mg, 4 mg and 9 mg tablets (emc products 102344, 102345, 102346). NICE search on 10 September 2026: the appraisal Oral semaglutide for managing overweight and obesity [ID6188] (GID-TA11149) was discontinued on 12 May 2025 because the company informed NICE it would not provide an evidence submission. NICE TA875 (2023) was read for the injectable product only. Summarised 10 September 2026.

NICE guidance position
No published NICE technology appraisal covers Wegovy tablets. NICE's project page for Oral semaglutide for managing overweight and obesity [ID6188] records that the company informed NICE it would not provide an evidence submission, and the appraisal was discontinued on 12 May 2025.
NICE TA875 (Semaglutide for managing overweight and obesity, 2023) appraised the injectable product and does not mention the tablet formulation; its eligibility, 2-year maximum and 5 percent stopping rule are therefore not NICE recommendations for the tablets.

Licensed indication (SmPC section 4.1)
Wegovy tablets are indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management, including weight loss and weight maintenance, in adults with an initial BMI of 30 kg/m2 or more (obesity), or 27 kg/m2 to less than 30 kg/m2 (overweight) in the presence of at least one weight-related comorbidity.
The safety and efficacy of oral semaglutide in children and adolescents below 18 years have not been established.

Dose escalation (SmPC section 4.2)
The starting dose is 1.5 mg once daily. After one month at this dose, the escalation steps are 4 mg, 9 mg and 25 mg once daily, with a minimum duration of 1 month at each dose level.
The dose should be escalated until 25 mg (the maintenance dose); if needed, the dose can be maintained at the previous dose level.
The maximum recommended single daily dose is 25 mg. Oral semaglutide should always be used as only one tablet per day; more than one tablet a day must not be taken to achieve the effect of a higher dose.
Missed dose: the missed dose should be skipped and the next dose taken the following day.
Patients treated with semaglutide injection 2.4 mg once weekly can be transitioned to semaglutide 25 mg tablets once daily, starting the tablets one week after the last injection; the SmPC notes that the effect of switching cannot easily be predicted because oral semaglutide displays higher pharmacokinetic variability in absorption than the injection.
Semaglutide should not be used in combination with other GLP-1 receptor agonist products. When initiating semaglutide in patients with type 2 diabetes, consider reducing the dose of concomitant insulin or insulin secretagogues (such as sulfonylureas) to reduce the risk of hypoglycaemia.

Oral administration rules (SmPC section 4.2)
Wegovy is a tablet for once-daily oral use.
It should be taken on an empty stomach after a recommended fasting period of at least 8 hours.
It should be swallowed whole with a sip of water (up to half a glass of water, equivalent to 120 mL). Tablets should not be split, crushed or chewed, as it is not known whether this affects absorption.
Patients should wait at least 30 minutes before eating, drinking or taking other oral medicinal products; waiting less than 30 minutes decreases the absorption of semaglutide.
Adherence to the dosing regimen is recommended for optimal effect. Section 4.4 adds that if the treatment response is lower than expected, the prescriber should be aware that absorption of oral semaglutide may be variable.

Special populations (SmPC section 4.2)
No dose adjustment is required based on age; experience in patients aged 85 years or over is limited.
No dose adjustment is required for mild or moderate renal impairment; semaglutide is not recommended in severe renal impairment (eGFR below 30 mL/min/1.73m2) including end-stage renal disease.
No dose adjustment is required for mild or moderate hepatic impairment, where it should be used cautiously; semaglutide is not recommended in severe hepatic impairment.

Contraindications (SmPC section 4.3)
Hypersensitivity to the active substance or to any of the excipients listed in section 6.1. No other contraindications are listed.

Warnings and precautions (SmPC section 4.4)
Gastrointestinal effects and dehydration: nausea, vomiting and diarrhoea may cause dehydration, which in rare cases can lead to deterioration of renal function; consider this in patients with impaired renal function and advise patients to take precautions to avoid fluid depletion. Section 4.8 adds that patients with moderate renal impairment (eGFR 30 to below 60) may experience more gastrointestinal effects.
Aspiration with general anaesthesia or deep sedation: pulmonary aspiration has been reported in patients on GLP-1 receptor agonists; the increased risk of residual gastric content due to delayed gastric emptying should be considered before such procedures.
Acute pancreatitis: not studied in patients with a history of pancreatitis, use with caution. Acute pancreatitis, including necrotising and fatal cases, has been reported with GLP-1 receptor agonists; inform patients of the symptoms (persistent,

severe abdominal pain) and to seek immediate medical attention; discontinue if suspected and do not restart if confirmed. Raised pancreatic enzymes alone are not predictive.
NAION: epidemiological data may indicate an increased risk; patients reporting sudden loss of vision (including partial loss) should be urgently referred for ophthalmological examination, and semaglutide discontinued if NAION is confirmed.
Gastroparesis: use with caution; not recommended if gastroparesis is severe.
Semaglutide must not be used as a substitute for insulin in patients with diabetes.
Hypoglycaemia: semaglutide lowers blood glucose and can cause hypoglycaemia; patients should know the signs and symptoms. Combination with a sulfonylurea or insulin may increase the risk, which can be lowered by reducing the sulfonylurea or insulin dose.
Diabetic retinopathy: increased risk of complications has been observed in patients with diabetic retinopathy treated with insulin and semaglutide; monitor closely. There is no experience in type 2 diabetes with uncontrolled or potentially unstable diabetic retinopathy.
Populations not studied: no experience in congestive heart failure NYHA class IV; limited experience in patients aged 85 years or more.
Sodium: the 1.5 mg, 4 mg and 9 mg tablets are essentially sodium-free; the 25 mg tablet contains 23 mg sodium, equivalent to 1 percent of the WHO recommended maximum daily intake.
Traceability: the name and batch number of the administered product should be clearly recorded.

Interactions (SmPC section 4.5)

Semaglutide delays gastric emptying, which may influence the absorption of other oral medicinal products.
Levothyroxine: total thyroxine exposure (adjusted for endogenous levels) increased by 33 percent after a single dose; monitoring of thyroid parameters should be considered when semaglutide is given with levothyroxine.
Warfarin and other coumarin derivatives: exposure and INR were not affected in a clinically relevant manner, but decreased INR has been reported with acenocoumarol, and frequent INR monitoring is recommended on initiation.
Rosuvastatin: AUC increased by 41 percent, not considered clinically relevant given its wide therapeutic index.
Digoxin, oral contraceptives (ethinylestradiol and levonorgestrel), metformin and furosemide: no clinically relevant change in exposure.
Omeprazole did not change semaglutide exposure to a clinically relevant degree.
When semaglutide was taken at the same time as five other tablets, semaglutide AUC fell by 34 percent and Cmax by 32 percent; patients should wait 30 minutes after semaglutide before taking other oral medicinal products.
Interactions with medicines of very low bioavailability (1 percent) have not been evaluated.

Pregnancy, breastfeeding and contraception (SmPC section 4.6)

Women of childbearing potential are recommended to use contraception when treated with semaglutide.
Semaglutide should not be used during pregnancy; if a patient wishes to become pregnant, or pregnancy occurs, it should be discontinued, and it should be stopped at least 2 months before a planned pregnancy because of its long half-life.
No measurable semaglutide was found in breast milk, but the absorption enhancer salcaprozate sodium and some of its metabolites were present at low concentrations; as a risk to a breast-fed child cannot be excluded, Wegovy should not be used during breast-feeding.

Driving (SmPC section 4.7)

No or negligible influence on the ability to drive or use machines, but dizziness may occur mainly during dose escalation, and patients on a sulfonylurea or insulin should take precautions against hypoglycaemia while driving.

Adverse effects (SmPC section 4.8)

The safety profile of the tablets in the 64-week OASIS 4 trial (204 patients exposed) was consistent with semaglutide injection, except that dyspepsia had a higher frequency category with the tablets.
Very common (1 in 10 or more): headache, vomiting, diarrhoea, constipation, dyspepsia, nausea, abdominal pain, fatigue.
Common (1 in 100 to 1 in 10): hypoglycaemia in patients with type 2 diabetes, dizziness, dysaesthesia, dysgeusia, diabetic retinopathy in type 2 diabetes, gastritis, gastro-oesophageal reflux disease, eructation, flatulence, abdominal distension, cholelithiasis, hair loss.
Uncommon (1 in 1,000 to 1 in 100): hypoglycaemia in patients without type 2 diabetes, increased heart rate, hypotension, orthostatic hypotension, acute pancreatitis, delayed gastric emptying, urolithiasis, increased amylase, lipase and bilirubin.
Rare: anaphylactic reaction, angioedema, hip fracture. Very rare: NAION. Not known: intestinal obstruction.
In OASIS 4, nausea occurred in 46.6 percent of patients on the tablets (18.6 percent placebo), vomiting in 30.9 percent (5.9 percent), diarrhoea in 17.6 percent (8.8 percent) and constipation in 20.1 percent (9.8 percent); gastrointestinal events led to permanent discontinuation in 3.4 percent.
Cholelithiasis was reported in 2.5 percent of patients on the tablets (1 percent placebo); hair loss in 6.4 percent (2.0 percent placebo), all mild or moderate; dysaesthesia-type events in 4.9 percent (none on placebo); mean heart rate rose by 2 beats per minute.
Large epidemiological studies suggest an approximately two-fold increase in the relative risk of NAION with semaglutide in adults with type 2 diabetes, about one additional case per 10,000 person-years.
Overdose may cause gastrointestinal disorders leading to dehydration; observe and give supportive treatment. The product carries a black triangle and suspected adverse reactions should be reported via the MHRA Yellow Card scheme.

What the sources do not say

No published NICE technology appraisal exists for Wegovy tablets; the NICE appraisal ID6188 was discontinued and no NICE eligibility criteria, BMI thresholds, ethnic BMI adjustments, treatment duration or stopping rule apply to the tablets from NICE.
Unlike the injection SmPC, the Wegovy tablets SmPC section 4.2 contains no rule to review treatment if less than 5 percent of body weight has been lost after 6 months.
The tablets SmPC has no separate gallbladder disease warning in section 4.4 (gallbladder events are listed in section 4.8 only), and it does not mention suicidal ideation or behaviour.
The SmPC text retrieved ended within section 5, so storage and shelf life (section 6) are not summarised here.

Patient Group Direction for the supply of oral semaglutide 1.5mg, 4mg, 9mg and 25mg tablets for weight management in adults aged 18 to 85 years.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for the product supplied, the BNF and applicable national guidance.
- All users must be competent in weight management consultation, BMI calculation and the recognition of red flags requiring referral.
- All users must be able to explain the empty-stomach administration requirements and check at each visit that they are being followed, because absorption depends on them.
- All users must know how to obtain and verify documentary evidence of a previous injection dose before switching a patient from the injection.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts, including the strengthened GLP-1 pancreatitis warning of 29 January 2026.
- All users must understand capacity and consent and be aware of the Mental Capacity Act 2005.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Wegovy (semaglutide) tablets are a UK-licensed oral GLP-1 receptor agonist indicated as an adjunct to a reduced-calorie diet and increased physical activity for weight management, including weight loss and weight maintenance, in adults with an initial BMI of 30 kg/m2 or above (obesity), or 27 kg/m2 to below 30 kg/m2 (overweight) in the presence of at least one weight-related comorbidity, for example hypertension, dyslipidaemia, obstructive sleep apnoea, cardiovascular disease or type 2 diabetes. This medicine is subject to additional monitoring (black triangle); report all suspected adverse reactions via the MHRA Yellow Card scheme.
Inclusion criteria	● Adults aged 18 to 85 years inclusive. ● BMI 30 kg/m2 or above, or BMI 27 to below 30 kg/m2 with at least one weight-related comorbidity. ● **Baseline weight measured and recorded at initiation, and available at every subsequent visit.** ● Willing to follow a reduced-calorie diet and increase physical activity in line with the agreed lifestyle plan. ● Initial assessment completed and documented. ● **Able to follow the empty-stomach administration requirements: a fast of at least 8 hours before the dose, and a 30 minute wait afterwards before food, drink or other oral medicines.** ● Informed consent obtained, including the side-effect profile and treatment expectations. ● **Where switching from the injection: documented evidence of the current injection dose obtained and recorded. See the switching row below.**

	● No exclusion criteria present.
Exclusion criteria	● Under 18 years of age. ● Over 85 years of age. This is a Get Real Health position, not a licence restriction; the SPC sets no upper age limit and records only that experience above 85 is limited. Refer to a specialist if treatment is being considered. ● **Pregnancy, breastfeeding, or planning pregnancy. Effective contraception is required throughout. Semaglutide must be discontinued at least 2 months before a planned pregnancy because of its long half-life, and stopped immediately if pregnancy occurs or is suspected (SPC section 4.6).** ● Known hypersensitivity to semaglutide or to any of the excipients. ● Personal or family history of medullary thyroid carcinoma, or Multiple Endocrine Neoplasia syndrome type 2. ● History of pancreatitis, acute or chronic. ● Severe gastrointestinal disease, including gastroparesis or a severe persistent gastrointestinal disorder. ● Current cholelithiasis or cholecystitis, or cholecystectomy within the last 3 months. ● Obesity caused by an endocrinological disorder. Where the patient was already overweight before that diagnosis, this exclusion may not apply. ● **CONCURRENT USE OF ANY OTHER GLP-1 RECEPTOR AGONIST, OR OF ANY INSULIN SECRETAGOGUE (SULFONYLUREA OR MEGLITINIDE), FOR ANY INDICATION. Ask specifically whether the patient takes anything for diabetes, and name the products. Several GLP-1 medicines are licensed for type 2 diabetes as well as for weight management: orforglipron for both, oral semaglutide separately for diabetes at 3mg, 7mg and 14mg, and semaglutide and tirzepatide injections in diabetes. A patient does not always think of a diabetes medicine as the same kind of drug.** ● Type 1 diabetes mellitus. ● Diabetic retinopathy. Treatment may worsen retinopathy; defer or refer to a specialist. ● Insulin-treated diabetes. Refer: a pharmacy weight-management service cannot manage insulin dose reduction. ● Severe renal impairment (eGFR below 30 mL/min/1.73 m2) or end-stage renal disease. ● Severe hepatic impairment. ● Known heart failure with reduced ejection fraction below 40%. ● Active eating disorder: anorexia nervosa, bulimia, or binge-eating disorder under specialist care. ● BMI below the inclusion threshold. ● **Switching from the injection where documented evidence of the current injection dose cannot be obtained.** ● Patients who, in the clinical judgement of the healthcare professional, are not suitable for the medicine.
Switching from semaglutide injection	**DOCUMENTED EVIDENCE IS REQUIRED. Patient self-report of the injection dose is not sufficient and must not be relied on.** **Acceptable evidence, one of:** ● A dispensing label, prescription, or repeat slip showing the strength. ● A record from the supplying clinic or pharmacy, including a GRH consultation record.

	● The patient's own pen or carton, seen by the pharmacist, with the strength legible. ● Written or verbal confirmation from the prescriber, recorded with the date and the name of the person who gave it. **Record which of these was seen, and the strength it showed. A photograph on a phone is acceptable where the strength is legible.** **Where the documented dose is 2.4mg once weekly:** ● Start Wegovy tablets at 25mg once daily, one week after the last injection. **Where the documented dose is LOWER than 2.4mg once weekly (0.25mg, 0.5mg, 1mg or 1.7mg):** ● **Do NOT start at 25mg. There is no licensed switch from these doses and 25mg would be a substantial jump.** ● **Either continue the injection under the original prescriber until 2.4mg is reached and then switch as above, or start the tablets as a new initiation at 1.5mg once daily, one week after the last injection, and titrate monthly through 4mg and 9mg to 25mg.** ● Explain to the patient that starting again at 1.5mg is not a step backwards but the only safe route, and that the titration protects them from gastrointestinal effects. ● This lower-dose pathway is a Get Real Health practice decision, not a licensed instruction. Record it as such. **Where the injection was stopped more than 2 months ago, treat as a new initiation and apply the BMI inclusion criteria afresh.**
Review and the stopping rule	**ONE RULE. The rule is now:** **Reassess whether to continue where the patient has lost less than 5% of their BASELINE body weight after 6 months at their established dose.** ● "Established dose" means 25mg once daily where the patient reached it. ● **Where the patient did not reach 25mg because of tolerability, it means the highest dose they have tolerated for at least 3 consecutive months. A patient who never reaches 25mg is not thereby exempt from review, which was the practical effect of the "maintenance dose" wording.** ● **The 5% is calculated from the recorded baseline weight, not from the weight at the last visit. Where no baseline was recorded, one must be established and the 6 months run from that point.** **At every visit:** ● Reassess clinical benefit, tolerability and progress against the agreed target weight. ● **Record weight, and the percentage change from baseline.** ● Confirm the empty-stomach administration requirements are being followed, since absorption depends on them. ● When the patient reaches their target weight, discuss whether to continue to maintain it.
Cautions	Counsel on gradual dose escalation and gastrointestinal side effects. Consider delaying titration or reducing to the previous dose if significant symptoms occur. **ACUTE PANCREATITIS. The MHRA strengthened the class warning for GLP-1 receptor agonists on 29 January 2026 after reports of necrotising and fatal cases. Make the red flag explicit to the patient: severe, persistent abdominal pain, often radiating to the back, means stop the medicine and seek urgent medical attention the same day.** **History of suicidal ideation, or active severe mental illness. Ensure**

	appropriate psychiatric oversight is in place, monitor mood at review, and refer if there is any concern. Do not supply where oversight is absent and concern exists. Non-arteritic anterior ischaemic optic neuropathy has been reported with semaglutide. Advise the patient to seek urgent review for any sudden loss of vision, and discontinue if it is confirmed. Mild to moderate renal impairment: monitor for dehydration secondary to gastrointestinal side effects. Delayed gastric emptying may reduce the absorption of oral medicines, especially those with a narrow therapeutic index. On starting semaglutide in a patient on warfarin, frequent INR monitoring is recommended. Decreased INR has been reported with acenocoumarol, so the same applies to other coumarins. **Levothyroxine: oral semaglutide increases levothyroxine exposure by about a third (AUC increased 33%). Monitor thyroid function when the two are taken together, and make sure the patient keeps the 30 minute separation, which matters more here than with most co-medicines.** **Pulmonary aspiration has been reported in patients on GLP-1 receptor agonists undergoing general anaesthesia or deep sedation. The increased risk of residual gastric content from delayed gastric emptying should be considered before any such procedure. Advise the patient to tell any anaesthetist that they take this medicine.** For patients who take oral HRT: the SmPC reports no clinically relevant change in exposure for an ethinylestradiol and levonorgestrel oral contraceptive, but has no data on HRT products; non-oral HRT such as a patch, gel or levonorgestrel intrauterine device may be considered. Tachycardia has been reported. In a patient with a pre-existing raised heart rate, use with caution and seek specialist advice first. Discontinue and seek advice for a clinically relevant sustained rise in resting heart rate. Type 2 diabetes on metformin, an SGLT2 inhibitor or a DPP-4 inhibitor only: no dose adjustment is needed, but inform the GP. Any sulfonylurea, meglitinide or insulin EXCLUDES (see exclusion criteria); there is no GP-monitored route for those patients under this PGD. Risk of dehydration. Counsel on adequate fluid intake during gastrointestinal side effects. The 25mg maintenance tablet contains 23mg of sodium. The lower strengths are essentially sodium free. Rarely material, but worth knowing for a patient on a sodium restricted diet.
Actions if the patient is excluded or declines	● Discuss the reason for exclusion and make sure the patient understands it. ● Advise on alternatives and how to access them: the GP, a specialist weight management service, or a lifestyle programme. ● Where the exclusion relates to an undiagnosed or unmanaged comorbidity, recommend GP review. ● **Where the exclusion is that injection-dose evidence could not be obtained, tell the patient exactly what to bring, so the appointment can be rebooked rather than abandoned.** ● Document the advice given and the decision reached. Inform or refer to the GP as appropriate.

The medicine

Name, form and strength	Wegovy (semaglutide) tablets: 1.5mg, 4mg, 9mg and 25mg. Black triangle medicine subject to additional monitoring. Record the product name and batch number for traceability.

Legal category	POM.
Storage	Store in the original package below 30 degrees Celsius to protect from moisture. Keep the container tightly closed.
Route and method of administration	• Oral, once daily, on an empty stomach after a fast of at least 8 hours, with no more than half a glass of water, about 120 mL. • **Wait at least 30 minutes before food, drink or other oral medicines.** • Swallow the tablet whole. Do not split, crush or chew. • **Only one tablet per day. Never combine tablets to approximate a higher dose.**
Dose and frequency	• Start at 1.5mg once daily for one month. Escalate monthly through 4mg and 9mg to the maintenance dose of 25mg once daily, with a minimum of one month at each step. The dose may be held at the previous level if needed. • Maximum dose 25mg once daily. • **Missed dose: skip it and take the next dose the following day. Never take two tablets in a day to make up a missed dose (SPC section 4.2).** • Where several consecutive doses have been missed, use clinical judgement about restarting at a lower step and re-escalating. That is a practice decision rather than a licensed instruction. • For significant gastrointestinal symptoms during titration, consider delaying a dose increase or dropping to the previous dose until symptoms improve. • To recommence after stopping, titrate again from the lowest dose. Apply the BMI inclusion criteria afresh if more than 2 months have passed since discontinuing. • **Switching from the injection: see the switching row above. Documented evidence of the current injection dose is required.**
Quantity to be supplied	• One calendar pack of 30 tablets at the appropriate strength, giving one month of treatment. • Supply one month of treatment per patient appointment. • **This PGD does not allow additional medicine to be supplied to enable a patient to stock up.**
Adverse effects	Very common: nausea, diarrhoea, vomiting, constipation and abdominal pain. Common: dyspepsia, eructation, flatulence, gastro-oesophageal reflux, gallstones, headache, fatigue, dizziness, hypoglycaemia when used with a sulfonylurea or insulin, hair loss and tachycardia. Uncommon or rare: acute pancreatitis, cholecystitis, hypersensitivity reactions including anaphylaxis, acute kidney injury usually secondary to dehydration, and non-arteritic anterior ischaemic optic neuropathy. Refer to the current SPC for full safety information.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • Name of the healthcare practitioner, and registration number. • Name and brand of the medicine, and the batch number. • Date of supply, dose, form, route and quantity supplied. • **BASELINE WEIGHT, recorded at initiation and carried forward at**

	every visit thereafter. ● **Height, weight and BMI at this visit, the percentage change from baseline, and the target weight agreed.** ● **Where switching from the injection: what documentary evidence of the injection dose was seen, and what strength it showed.** ● **The current established dose, and where it is below 25mg, how long the patient has been on it.** ● Confirmation that the empty-stomach requirements are being followed. ● Advice given, including advice given if the patient is excluded or declines. ● Where no supply is made, the record still carries the patient details, the exclusion or the reason, and the advice given. ● Details of any adverse drug reactions and the actions taken. ● That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Adults aged 18 and over: keep for 8 years.

Information for the patient

Written information	Supply the patient information leaflet provided with the medicine, together with written lifestyle, diet and physical activity advice and the agreed target weight.
Counselling	● **Take one tablet a day on an empty stomach, after at least 8 hours without food, with no more than half a glass of water. Wait 30 minutes before anything else, including other tablets.** ● Swallow whole. Do not split, crush or chew. Never take two in a day. ● The medicine works alongside diet and activity, not instead of them. ● Nausea and other stomach effects are common at first and usually settle. Drink enough fluid. ● **Tell any anaesthetist, dentist or surgeon that you take this medicine before any procedure with sedation or a general anaesthetic.**
Follow-up and urgent symptoms	**Seek urgent medical attention the same day for:** ● **Severe, persistent abdominal pain, often going through to the back. Stop the medicine.** ● Persistent vomiting with signs of dehydration. ● Yellowing of the skin or the whites of the eyes. ● Sudden loss of vision in one or both eyes. ● A sustained rise in resting heart rate. Attend for review as agreed. Treatment will be reassessed if less than 5% of your starting weight has been lost after 6 months at your established dose.

Appendix 1: Key references

- Summary of Product Characteristics for the product supplied, current version.
- NICE guidance on obesity management and on the relevant medicine.
- MHRA Drug Safety Update, GLP-1 receptor agonists and acute pancreatitis, 29 January 2026.
- Other MHRA Drug Safety Updates relevant to GLP-1 receptor agonists.
- British National Formulary.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions, https://www.nice.org.uk/guidance/mpg2

Authorisation

Version	v013
Supersedes	Version 012, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

GET REAL HEALTH

Change history to version 006

Later versions are recorded in the Version and change record at the end of this document.

Version	v006
Date	8 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. ● The stopping rule is now a single rule. Version 004 stated "6 months on the maintenance dose" in the cautions and in the patient follow-up advice, and "6 months on the maximum tolerated dose" in the monitoring section. It now reads: reassess where less than 5% of baseline body weight has been lost after 6 months at the established dose, being 25mg where reached, or the highest dose tolerated for at least 3 consecutive months where it was not. A patient who never reaches 25mg is no longer exempt from review. ● Baseline weight must be recorded at initiation and carried forward at every visit, and the 5% is calculated from it. The 5% test existed in version 004 but nothing required the baseline it depends on to be recorded. ● Switching from semaglutide injection now requires documented evidence of the current injection dose. Patient self-report is not sufficient. Four acceptable forms of evidence are listed and the one relied on must be recorded. ● A pathway is stated for patients switching from an injection dose below 2.4mg, which version 004 did not cover at all. Do not start at 25mg: either continue the injection to 2.4mg under the original prescriber, or start the tablets as a new initiation at 1.5mg and titrate. Marked in the document as a practice decision rather than a licensed instruction. ● The concurrent GLP-1 exclusion now reads "for any indication", closing the gap where a patient taking a GLP-1 for type 2 diabetes was not caught by an exclusion limited to weight management use. The correction notice issued on 7 September 2026 is incorporated and withdrawn. ● Requirement added to advise the patient to tell any anaesthetist, dentist or surgeon that they take this medicine before a procedure under sedation or general anaesthetic, the pulmonary aspiration point having previously appeared only in the cautions to the pharmacist. ● Records section expanded to require baseline weight, percentage change from baseline, the evidence seen for a previous injection dose, and the current established dose with its duration.

Previous versions

001	Development and issue of new PGD.
004, 21 August 2026	Reconciled against the UK SPC published after v003 was signed: levothyroxine interaction added, missed dose instruction corrected to the

	licensed wording, MHRA January 2026 pancreatitis warning added, and the tirzepatide clause removed from the pregnancy exclusion.

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....
.....	..... | ..... | ..... | .....

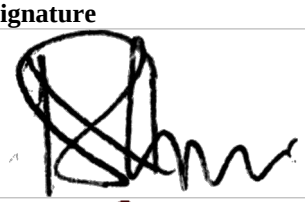
Version and change record
Version: v013
Issued: 30 September 2026
Supersedes: Version 012, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v012
Issued: 14 September 2026
Supersedes: Version 011, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v011
Issued: 11 September 2026
Supersedes: Version 010, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Inclusion BMI band stated as 27 to below 30 with a comorbidity, the SmPC phrasing used elsewhere in the document.
Concurrent-use exclusion names sulfonylureas and meglitinides, matching the cautions row.
HRT caution explains that the SmPC oral contraceptive finding does not cover HRT products, so it no longer reads as a contradiction.
Records row states what is recorded when no supply is made.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v010
Issued: 11 September 2026
Supersedes: Version 009, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v007
Issued: 9 September 2026
Supersedes: the previous signed version of this document, and any correction notice attached to it.

Change: Version 006 opened with a "three things changed in this version" block on page one, ahead of the clinical content, which is version administration in the body of a clinical document. It is removed. Every item in it was already in the change history and remains there. No clinical change.
Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.
Version: v008
Issued: 10 September 2026
Supersedes: Version 007, 9 September 2026
Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.
Version: v009
Issued: 10 September 2026
Supersedes: Version 008, 10 September 2026
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
One rule for insulin secretagogues and insulin. The previous version excluded every sulfonylurea or meglitinide user in one bullet, then excluded them only 'where the GP is unwilling to monitor' two bullets later, then made co-use a referral caution on the next page. The blanket exclusion stands; insulin-treated diabetes is now an exclusion in the same terms; the conditional exclusion and the caution are gone.

Signed on behalf of Get Real Health

Version v013, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.